Methotrexate: head and neck cancer

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Recurrent or advanced head and neck cancer · NCCP 00893 v1 · Document revised 14 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Recurrent, advanced or metastatic head and neck squamous cell cancer in people with comorbidities or unsuitable for combination chemotherapy.

My overall treatment

intravenous (IV) on Days 1, 8 and 15 every 28 days



Treatment

Methotrexate 40 mg/m² by intravenous bolus on Days 1, 8 and 15 of each 28-day cycle, until progression or unacceptable toxicity. There is no scheduled Day-22 dose. The clinician may increase to 50 mg/m² only if there has been no mucositis.

Review

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

A typical 28-day cycle

Day 1	Day 2–7	Day 8	Day 9–14	Day 15	Day 16–28
IV treatment	Days 2–7: between doses	IV treatment	Days 9–14: between doses	IV treatment	Days 16–28: between doses

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline preparation** NCCP	Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.
During	**Safety checks** NCCP	Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.
Response	**Cancer-specific checks** Individual specialist plan	Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant's plan; no routine universal tumour-marker panel is specified.

Between treatments

- Symptoms can occur on any day; use your written contact plan.
- Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.

Bring your medicines list, calendar and verified contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

Methotrexate

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

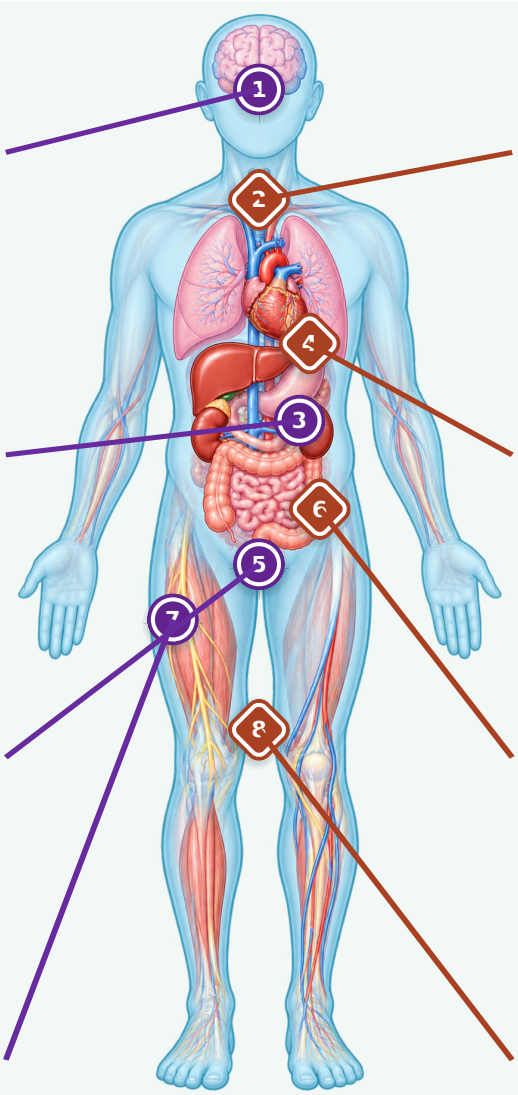
Methotrexate

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Blood counts and infection

Methotrexate

Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent oncology advice.



4 Breathing

Methotrexate

New cough or breathlessness needs prompt review; serious causes include infection or lung injury.

5 Sickness and bowels

Methotrexate

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

Methotrexate

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Skin, hands and feet

Methotrexate

Report painful palms or soles, rash or blistering early. Severe spreading rash or mouth/eye involvement needs urgent review.

8 Fertility and pregnancy

Methotrexate

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Methotrexate
IV on Days 1, 8 and 15 every 28 days
Phase: _____ Cycle plan: team to confirm
Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Recording spaces only; not a fixed course.

Cycle	Date / initials

My treatment information

Latest treatment date: _____
Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.